INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Lorlatinib therapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00570
Version 3

Once daily, continuously

Treatment stages

01 **Continuous daily lorlatinib**
One dose every day

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Lorlatinib aims to control ALK-positive advanced lung cancer, including disease in the brain where present. Your oncology team explains the likely benefit, treatment line and alternatives for your situation.

ALK TYROSINE KINASE INHIBITOR

Lorlatinib

Blocks abnormal ALK growth signals

ALK-positive cancer identified → Abnormal ALK signalling blocked → Cancer growth may be controlled, including in the brain

Lorlatinib blocks the abnormal ALK protein that helps some lung cancers grow. It can reach the brain, an important site of disease in ALK-positive lung cancer. It is selected using an ALK test. It can also affect healthy tissues, so blood tests and symptom checks remain important.

Report thinking, mood, speech, breathing or heart symptoms promptly and attend lipid, glucose and other monitoring.

Understanding the treatment

Confirm the setting and individual aim.

Why this medicine

EMA Lorviqua current product information and patient leaflet

Which indication

This guide covers lorlatinib alone for ALK-positive advanced non-small-cell lung cancer. The available Irish NCCP v3 document lists use after particular earlier ALK medicines. The current European licence also includes people who have not had an ALK inhibitor. The team must confirm your treatment setting and the current Irish protocol and access arrangements before prescribing; the source versions currently disagree.

NCCP 00570 v3 · official regimen · EMA Lorviqua current product information and patient leaflet

Evidence differs before and after earlier ALK treatment

A first-line randomised trial and separate later-line cohorts answer different questions.

First-line and later-line results are not interchangeable. Current evidence does not provide a personal prognosis or a direct comparison against every other ALK treatment.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact your oncology team promptly for changes in memory, thinking, mood, speech or behaviour, hallucinations, new or worsening breathlessness, cough, fainting, a slow or irregular heartbeat, severe abdominal pain, marked thirst or frequent urination. Severe confusion, unsafe behaviour, severe breathing difficulty or collapse needs emergency help. Do not wait for the next appointment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Checks during lorlatinib treatment

Monthly monitoring is not a chemotherapy cycle. Baseline coagulation is separate from routine monthly tests. Scans and extra checks follow the Consultant plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00570 v3 · official regimen · EMA Lorviqua current product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Lorlatinib therapy: treatment and checks

Your treatment pattern

Once daily, continuously

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ALK-positive advanced non-small-cell lung cancer (NSCLC) · specified later-line NCCP indication; separate licensed first-line context

Lorlatinib is taken once daily continuously, with no planned rest week. Treatment continues while it is helping and side effects remain acceptable; there is no fixed number of treatment cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First four weeks	Weeks 5–8	Ongoing treatment	Hold / change / completion
Treatment and benefit NCCP 00570 · SMPC	Confirm ALK-positive cancer, treatment line and access	Daily tablets; no rest week	Continue daily if authorised	Review benefit, harms and preferences	Team decides hold, switch or stop
Lipids and glucose NCCP 00570 · SMPC	Baseline lipids and glucose	Lipids at weeks 2 and 4	Lipids at week 8	Lipids thereafter as appropriate; glucose as indicated	Review lipid/ glucose medicines and recovery
Blood and pancreatic tests NCCP 00570 · SMPC	full blood count (FBC), kidney/liver profile, clotting, amylase/ lipase	FBC, kidney/ liver and pancreatic enzymes monthly	Continue monthly checks	Monthly or additional tests as indicated	Follow abnormal results and symptoms

Care / check	Before starting	First four weeks	Weeks 5–8	Ongoing treatment	Hold / change / completion
Heart and blood pressure NCCP 00570 · SMPC	electrocardiogram (ECG) and blood pressure (BP); left ventricular ejection fraction (heart pumping measurement) (LVEF) if cardiac risk warrants	BP at two weeks then at least monthly; ECG monthly	Continue ECG and BP monitoring	Earlier cardiac assessment for symptoms; LVEF if indicated	Restart only after source recovery criteria and specialist review
Symptoms and interactions NCCP 00570 · SMPC	Baseline cognition, mood, medicines and support needs	Report changes promptly; check adherence and interactions	Review central nervous system (CNS), breathing, nerve and metabolic effects	Symptoms can occur at any time; scans per Consultant plan	Follow unresolved effects and contraception intervals

What each check is for

1 Confirm source and setting

Before starting

Previous therapy changes the applicable indication.

Ask / act: Confirm ALK test, treatment line, prescription and baseline tests.

2 Early metabolic checks

Weeks 2, 4 and 8

Lipids can rise without symptoms.

Ask / act: Confirm who books, reviews and acts on results.

3



Monthly review

During continuous therapy

Organ function, conduction and symptoms may change.

Ask / act: Review dose, medicines, CNS/lung/cardiac symptoms and treatment benefit.

4 Hold or change

Whenever toxicity or progression requires

Restart criteria differ by toxicity.

Ask / **act:** Follow a written clinician plan; continue reporting symptoms and contraception advice.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily treatment, early checks and ongoing review

No patient dates are required. Follow your own appointment and prescription plan.

Treatment or check	Timing in this view
Prepare safely	**Before starting** Confirm ALK, setting, medicines and baseline tests
Daily lorlatinib	**First four weeks → Weeks 5–8 → Ongoing treatment** Once daily continuously; no scheduled rest week
Early lipid checks	**First four weeks → Weeks 5–8** Lipids at weeks 2, 4 and 8; blood pressure (BP) after two weeks
Ongoing monitoring	**First four weeks → Weeks 5–8 → Ongoing treatment → Hold / change / completion** Monthly blood tests and electrocardiogram (ECG); symptom and benefit review
Change or completion	**Hold / change / completion** Written hold/restart plan and follow-up of ongoing effects

If a dose is missed, take it when remembered if the next dose is at least four hours away; otherwise skip it. Do not take two doses together. If you vomit after a dose, do not take another tablet; take the next dose at the usual time. These instructions follow the current product leaflet.

Monitoring milestones are reference windows, not booked appointments.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Taking lorlatinib

Use your own prescription and hold/restart instructions.

Daily treatment

The usual starting dose is 100 mg once daily; use the dose on your prescription. Take it at about the same time each day, with or without food. Swallow whole; do not crush, chew or split tablets, or use damaged tablets. Avoid grapefruit and grapefruit juice.

Missed or vomited dose

If a dose is missed, take it when remembered if the next dose is at least four hours away; otherwise skip it. Do not take two doses together. If you vomit after a dose, do not take another tablet; take the next dose at the usual time. These instructions follow the current product leaflet.

Interactions

Avoid grapefruit and St John’s wort. Check all prescriptions, supplements and herbal products before any change; interactions can alter lorlatinib or other medicines.

Tests and important effects

Effects may occur between appointments.

Expected monitoring

Before starting: ALK testing, blood count, kidney and liver tests, clotting tests, cholesterol and triglycerides, glucose, pancreatic enzymes, blood pressure and an ECG. Lipids are checked at weeks 2, 4 and 8, then as needed. Blood count, kidney/liver tests, ECG and pancreatic enzymes are normally checked monthly. Blood pressure is checked after two weeks and at least monthly; glucose as needed. Scans follow your oncology plan.

NCCP 00570 v3 · official regimen

Thinking and mood

Ask someone you trust to help notice changes in thinking, mood, speech or behaviour, if you wish. Report changes early. Lorlatinib can affect driving and machine use, especially when CNS effects are present; discuss whether it is safe for you.

Urgent changes

Contact your oncology team promptly for changes in memory, thinking, mood, speech or behaviour, hallucinations, new or worsening breathlessness, cough, fainting, a slow or irregular heartbeat, severe abdominal pain, marked thirst or frequent urination. Severe confusion, unsafe behaviour, severe breathing difficulty or collapse needs emergency help. Do not wait for the next appointment.

Daily life and planning

Discuss practical, family and fertility concerns early.

Support and sickness

This oral regimen has minimal to low sickness risk. Use anti-sickness medicine only according to your own plan and report persistent vomiting or poor intake. A standard chemotherapy steroid or anti-sickness course is not automatically needed.

NCCP 00570 v3 · official regimen

Pregnancy and fertility

Lorlatinib may harm an unborn baby and make hormonal contraception less effective. Women who could become pregnant need highly effective non-hormonal contraception during treatment and for at least 35 days afterwards; discuss any unavoidable hormonal method and barrier protection. Men with partners who could become pregnant need effective contraception including a condom during treatment and for at least 14 weeks afterwards; condoms are also needed with a pregnant partner. Discuss fertility preservation before treatment. Do not breastfeed during treatment; agree when it can safely resume with the team.

Treatment decisions

NCCP 00570 v3 · official regimen

Practical support

Taking and replacing tablets

Cholesterol and other medicines

Cholesterol or triglycerides may rise without symptoms. Attend blood tests and take any prescribed lipid-lowering medicine. Ask the pharmacist before starting or stopping any medicine, supplement or herbal product, including St John’s wort. Do not change the lorlatinib dose yourself.

Sickness and daily wellbeing

Thinking, mood and practical support

Pregnancy, fertility and feeding

Taking tablets / attending treatment

Practical treatment details

Appointments and refills

Keep enough tablets for the prescribed plan.

Support for daily life

Bring a medicines list.

Ask about help with travel and work.

Questions to ask

Why is an ALK test needed?

Lorlatinib is intended for cancers with a specific ALK change. Your team checks the result and whether the treatment fits your previous medicines.

Why are cholesterol tests needed if I feel well?

Blood fats can rise without symptoms. Regular tests let the team treat this early. Raised cholesterol does not always mean lorlatinib must stop; your clinician uses the actual results and your overall condition.

What changes should my family or friends notice?

Changes in memory, concentration, mood, speech, behaviour or perceptions should be reported promptly. Agree with the team and someone you trust how changes will be noticed, if helpful.

Can I drive?

Lorlatinib can affect thinking and other CNS functions. Use caution and discuss driving or operating machinery, particularly if symptoms occur.

Does a dose reduction mean treatment has failed?

No. A hold or reduction may allow side effects to recover and reduce harm. The team reviews cancer control separately and explains the restart plan.

What belongs in the consent discussion?

Discuss your aim, expected benefits and uncertainty, reasonable alternatives including not proceeding, serious and lasting risks, daily practicalities, contraception, fertility, support needs and your right to reconsider.

Explain the plan back: Explain how you take lorlatinib and what happens after a missed or vomited dose.,Name the changes in thinking, mood, breathing or heartbeat that need prompt contact.,Confirm who books and reviews lipid, blood, BP and ECG checks.

Which treatment this guide covers This guide covers lorlatinib alone for ALK-positive advanced non-small-cell lung cancer. The available Irish NCCP v3 document lists use after particular earlier ALK medicines. The current European licence also includes people who have not had an ALK inhibitor. The team must confirm your treatment setting and the current Irish protocol and access arrangements before prescribing; the source versions currently disagree.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous lorlatinib · individual supportive care

Active medicines: lorlatinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP rates this oral regimen minimal to low emetogenic risk; follow local oral-systemic anticancer therapy (SACT) prevention/rescue policy. NCIS V8 describes parenteral antiemetics and is not copied as a daily oral schedule.
- No routine premedications are required. Lipid-lowering, blood pressure (BP)/glucose and symptom treatment are individual prescriptions reviewed for interactions.

- If a dose is missed, take it when remembered if the next dose is at least four hours away; otherwise skip it. Do not take two doses together. If you vomit after a dose, do not take another tablet; take the next dose at the usual time. These instructions follow the current product leaflet.
- Lorlatinib may harm an unborn baby and make hormonal contraception less effective. Women who could become pregnant need highly effective non-hormonal contraception during treatment and for at least 35 days afterwards; discuss any unavoidable hormonal method and barrier protection. Men with partners who could become pregnant need effective contraception including a condom during treatment and for at least 14 weeks afterwards; condoms are also needed with a pregnant partner. Discuss fertility preservation before treatment. Do not breastfeed during treatment; agree when it can safely resume with the team.
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antimicrobial prophylaxis or steroid course is specified in NCCP 00570.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Which treatment this guide covers This guide covers lorlatinib alone for ALK-positive advanced non-small-cell lung cancer. The available Irish National Cancer Control Programme (NCCP) v3 document lists use after particular earlier ALK medicines. The current European licence also includes people who have not had an ALK inhibitor. The team must confirm your treatment setting and the current Irish protocol and access arrangements before prescribing; the source versions currently disagree.

NCCP 00570 · VERSION 3

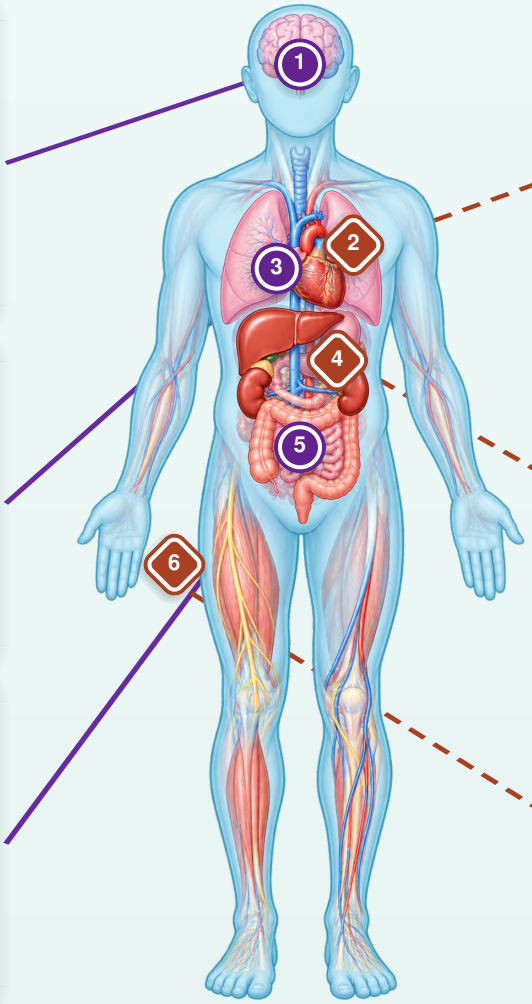
Lorlatinib therapy

Lorlatinib is taken once daily continuously, with no planned rest week. Treatment continues while it is helping and side effects remain acceptable; there is no fixed number of treatment cycles.

ALK-targeted tablets · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Thinking / mood / speech

Lorlatinib

Report changes in memory, behaviour, mood or perceptions promptly.

2 Lungs / breathing

Lorlatinib

New or worsening cough and breathlessness need urgent review.

3 Heart / blood pressure

Lorlatinib

Report fainting, dizziness, an irregular heartbeat or new swelling.

4 Blood fats / sugar

Lorlatinib

These can rise without symptoms; attend scheduled blood tests.

5 Abdomen / pancreas

Lorlatinib

Report severe or persistent abdominal pain, nausea or vomiting.

6 Hands / feet

Lorlatinib

Report numbness, tingling, weakness or changes in balance.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact your oncology team promptly for changes in memory, thinking, mood, speech or behaviour, hallucinations, new or worsening breathlessness, cough, fainting, a slow or irregular heartbeat, severe abdominal pain, marked thirst or frequent urination. Severe confusion, unsafe behaviour, severe breathing difficulty or collapse needs emergency help. Do not wait for the next appointment. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Thinking / memory / speech changes	☐	☐	☐	
Mood / behaviour / unusual perceptions	☐	☐	☐	
Breathlessness / cough	☐	☐	☐	
Dizziness / fainting / heartbeat changes	☐	☐	☐	
Swelling / weight change	☐	☐	☐	
Numbness / tingling	☐	☐	☐	
Thirst / passing urine more often	☐	☐	☐	
Abdominal pain / nausea	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Contact your oncology team promptly for changes in memory, thinking, mood, speech or behaviour, hallucinations, new or worsening breathlessness, cough, fainting, a slow or irregular heartbeat, severe abdominal pain, marked thirst or frequent urination. Severe confusion, unsafe behaviour, severe breathing difficulty or collapse needs emergency help. Do not wait for the next appointment. Emergency symptoms: 112 / 999.

Carry your treatment details

Which treatment this guide covers This guide covers lorlatinib alone for ALK-positive advanced non-small-cell lung cancer. The available Irish NCCP v3 document lists use after particular earlier ALK medicines. The current European licence also includes people who have not had an ALK inhibitor. The team must confirm your treatment setting and the current Irish protocol and access arrangements before prescribing; the source versions currently disagree.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Lorlatinib therapy

NCCP 00570 v3

Lorlatinib is taken once daily continuously, with no planned rest week. Treatment continues while it is helping and side effects remain acceptable; there is no fixed number of treatment cycles.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Contact your oncology team promptly for changes in memory, thinking, mood, speech or behaviour, hallucinations, new or worsening breathlessness, cough, fainting, a slow or irregular heartbeat, severe abdominal pain, marked thirst or frequent urination. Severe confusion, unsafe behaviour, severe breathing difficulty or collapse needs emergency help. Do not wait for the next appointment.

https://sactcheck.com/patient/00570/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.24 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00570 v3 · official regimen**
   https://healthservice.hse.ie/documents/6889/570_V3_Lorlatinib_therapy_.pdf
2. **EMA Lorviqua current product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/lorviqua-epar-product-information_en.pdf
3. **NCCP NCIS antiemetic medicines V8 (parenteral scope; no oral schedule copied)**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
4. **CROWN 7-year update, Annals of Oncology 2026: first-line lorlatinib versus crizotinib**
   https://www.annalsofoncology.org/article/S0923-7534%2826%2900876-8/fulltext
5. **Solomon et al., Lancet Oncology 2018: global phase 2 ALK cohorts**
   https://pubmed.ncbi.nlm.nih.gov/30413378/